

HERPES ZOSTER IN THE IMMUNOCOMPROMISED HOST

Except for postherpetic neuralgia (PHN), most serious complications of herpes zoster occur in immunocompromised individuals. These complications include skin necrosis and scarring, as well as cutaneous dissemination, which occurs in 25% to 50% of such patients. Approximately 10% of those with cutaneous dissemination develop widespread, often fatal, visceral involvement affecting the lungs, liver, and brain.

HIV-infected patients are particularly prone to multiple recurrences of herpes zoster as their disease progresses. Recurrences may involve the same or different dermatomes, including contiguous or non-contiguous ones. In patients with acquired immunodeficiency syndrome (AIDS), herpes zoster can be severe, with cutaneous and visceral dissemination. Additionally, these patients may develop chronic verrucous, hyperkeratotic, or ecthymatous skin lesions due to acyclovir-resistant varicella-zoster virus (VZV).

DIAGNOSIS OF VARICELLA

Varicella is typically diagnosed based on the characteristic appearance and progression of the rash (see Fig. 194-4), especially when there is a history of exposure within the preceding 2 to 3 weeks.

Disseminated herpes zoster may mimic varicella when VZV spreads widely from a small, painless zoster lesion or from an affected sensory ganglion, even in the absence of a clear dermatomal rash. This presentation is not uncommon in profoundly immunosuppressed, VZV-seropositive individuals.

Disseminated herpes simplex virus (HSV) infections can also resemble varicella; however, lesions are typically concentrated around the site of primary or recurrent infection (e.g., mouth or genitalia), and patients may exhibit significant systemic toxicity or encephalitis.

The differential diagnoses of varicelliform rashes are listed in Box 194-1. The morphology, distribution, and evolution of lesions, along with a detailed

epidemiologic history, usually allow differentiation from varicella. When diagnostic uncertainty persists, laboratory confirmation should be obtained.

DIAGNOSIS OF HERPES ZOSTER

In the pre-eruptive phase, the prodromal pain of herpes zoster is often mistaken for other causes of localized pain. Once the rash appears, the characteristic dermatomal distribution, along with associated dermatomal pain or discomfort, typically makes the diagnosis clinically evident (see Figs. 194-5 and 194-6).